Approve a 90-day pilot

One phone number, one nurse workflow, one pulmonology clinic. A lung cancer pathway measured from the first call to specialist care.

THE ASK

We ask one wellbeing county to approve a 90-day pilot with one phone number, one nurse workflow and one pulmonology clinic, to name a clinical sponsor, and to supply a baseline extract of referral and pathway timestamps.

The pilot tests three proposed local targets. The CT and visit figures are pilot targets derived from published county guidance, not numbers those pages print. The callback target of 2 working days is Polku's proposal, not a national or Nordic standard.

THE GAP

Filha's national lung cancer programme states that 65-70 % of lung cancers are only found at an advanced stage. Filha cites Knuuttila 2021 (Duodecim Oppiportti), not the Cancer Registry. [1]

In 2025, 76.8 % of referrals to specialised care in Finland were processed within 3 days. Among wellbeing counties with data that year, the share ran from 68.9 % in Etelä-Pohjanmaa to 88.7 % in Kanta-Häme. Etelä-Pohjanmaa was 78.1 % in 2021. This indicator covers all specialities and is reported by the producer of care, so it does not measure the lung pathway. [2]

We found no public county-level lung cancer access-time series. Counties may hold internal data. Denmark and Sweden publish pathway times per region. [6]

HOW IT WORKS

Call
The caller describes symptoms on one phone number. Patients never use a screen.

Typed symptoms
The language model extracts typed symptoms, with the caller's words attached. It does not diagnose or triage.

Cited rules
Deterministic rules match published Nordic referral criteria, each with its source.

Nurse confirms
The nurse gets one task with a rule-proposed time, and accepts or dismisses it.

Callback
On accept, the agent calls the patient back and books the next step.

Handoff strip
The strip shows who holds the patient. Every record access is logged.

Red flags return call 112 from the engine, whatever the model says. A human makes every clinical decision.

THREE PROPOSED TARGETS

2
WORKING DAYS
Nurse callback
Polku's proposal, not a national or Nordic standard. Nearest published precedent: Pirha usually processes lung referrals in 1-2 working days. [3]

POLKU PROPOSAL

7
DAYS
CT after referral processed
A pilot target derived from Pirha's wording, not a number that page prints: "CT usually within a week of the referral being processed." [3]

DERIVED FROM COUNTY GUIDANCE

21
DAYS
First pulmonology visit
A pilot target derived from Keski-Suomi (HYVAKS) wording, not a printed "21 days": "specialist visit goal of 2-3 weeks, typically about a month." [4]

DERIVED FROM COUNTY GUIDANCE

RECOMMENDED CARE ACTIONS

Nurse callback queue
Owner: health centre nurse lead. **Precedent:** Pirha usually processes lung referrals in 1-2 working days. [3] **Metric:** share of cases with a nurse callback within 2 working days.

Direct CT route on criteria match
Owner: primary-care lead, with radiology. Roadmap measure 2.5.2. [5] **Precedent:** Capital Region of Denmark, CT within 7 calendar days of the referral being received. [7] Norway: CT "without delay" on clinical suspicion, even if the X-ray is normal. [8] **Metric:** share of confirmed referrals with CT within the 7-day pilot target.

Protected CT slots
Owner: radiology. **Precedent:** no published Finnish protected-slot rule was found. The 2 slots a week are a planning assumption, aimed at Pirha's usual one-week CT. [3] **Metric:** share of CTs done within the 7-day pilot target.

What is measured. Handoff timestamps: first call to nurse, referral to CT, and CT to first pulmonology visit, and the share of cases meeting each of the three targets. Polku is the measurement and handoff layer that shows whether the actions work. The baseline extract tests the handoffs before any capacity changes.

What it takes. Planning assumptions, not a forecast: about 5 nurse hours a week (10 cases x 30 min) and 2 protected CT slots a week. A clinical sponsor, and a baseline extract of referral and pathway timestamps. No saving is forecast.

STRATEGY MEASURES SERVED

2.5.2 symptom-based referral criteria. 3.1.2 pathways with target access times. 3.5.3 digital pathways. Goal 3 measure: the share of patients who start treatment within a county-agreed target. [5]

HONESTY NOTES

Patients and pathway clocks in the demo are SYNTHETIC. The Sotkanet and Filha figures above are real public aggregates. Mobile ID is simulated. The voice runs on ElevenLabs today, and EU-only processing is a design goal, not the current state. The callback rings in the browser. This is a research prototype, not for clinical use, and it needs a regulatory assessment before deployment. Clinical effect, workload and equity effects are unproven until a pilot measures them.

Introduce us to one wellbeing county.

SOURCES

[1] Filha, Valtakunnallinen keuhkosyöpäohjelma 2026-2035, preprint 18.05.2026, citing Knuuttila 2021 (Duodecim Oppiportti). https://www.filha.fi/wp-content/uploads/2026/05/Valtakunnallinen-keuhkosyopaohjelma-2026-2035_180526_preprint.pdf

[2] THL Sotkanet indicator 13, updated 5.6.2026 (REST API values, 2021 and 2025, gender total). https://sotkanet.fi/sotkanet/en/metadata/indicators/13

[3] Pirkanmaan hyvinvointialue, Keuhkosyöpäpotilaan hoitopolku, updated 19.8.2026. https://www.pirha.fi/asiakkaalle/hoito-ja-palvelupolut/keuhkosyopapotilaan-hoitopolku

[4] Keski-Suomen hyvinvointialue (HYVAKS), Keuhkosyöpäpotilaan hoitopolku. https://www.hyvaks.fi/asiakkaana/hoitopolut/keuhkosyopapotilaan-hoitopolku

[5] National cancer strategy 2026-2035 p.5 and implementation roadmap 2026-2031 pp.52-57 and 73-74. https://files.hackjunction.app/resources/Kansallinen_syopastrategia_2026_2035_FINAL.pdf

[6] Search scope and limits: docs/lung-pathway-sources.md in https://github.com/3nd3r1/polku

[7] Region Hovedstaden, Lungekræft GP guidance, updated 23.03.2022. https://www.sundhed.dk/sundhedsfaglig/information-til-praksis/hovedstaden/almen-praksis/patientbehandling/patientforloeb/forloebsbeskrivelser-icpc/r-luftveje/lungekraeft/

[8] Helsedirektoratet, Inngang til pakkeforløp for lungekreft (NO-F). https://www.helsedirektoratet.no/nasjonale-forlop/lungekreft/inngang-til-pakkeforlop-for-lungekreft